Abdominal (GI) Examination — OSCE Checklist

Deakin MD2 · RG5 · Time: 14 min

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional, verbalised or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **General → Vitals → Hands → Arms → Face → Chest → Abdomen (inspect, palpate, percuss, auscultate) → Hernias → Complete**. *Watch the patient's face throughout and examine any painful area* **last**.

1. Introduction

☐	Wash hands; introduce self and the exam; confirm patient ID
☐	Explain and gain consent; ask about pain (examine that area last)
☐	Offer a chaperone; privacy (curtains); quiet, well-lit, warm room
☐	Position: lying flat on **one pillow**, arms by sides
☐	Expose abdomen; cover upper and lower body with 2 blankets (ideally nipples to mid-thigh †)
☐	Equipment: stethoscope, tape measure, torch

2. Examination

	Step	What to do / say	Abnormal
General inspection			
☐	Jaundice	Skin and sclera	Liver disease, haemolysis, obstruction
☐	Skin	Discoloration, pigmentation, rashes, acanthosis nigricans	
☐	Pain and position	Lying very still / writhing / sitting forward	Peritonitis / renal colic / pancreatitis
☐	Body habitus	Wasting or obesity	Malignancy, malabsorption
☐	Mental state	Confusion	Hepatic encephalopathy
Vital signs			
☐	Pulse (manual)	Rate, rhythm, character	Tachycardia: sepsis, bleeding
☐	BP lying and standing	Postural drop	Hypovolaemia (GI bleed)
☐	RR, temp, SpO_2, BGL	Full set; height and weight if possible	
Hands (both)			
☐	Nails	Leukonychia, clubbing	Hypoalbuminaemia; IBD, cirrhosis
☐	Palms	Pale palmar creases; palmar erythema	Anaemia; liver disease (may be normal)
☐	Dupuytren's contracture	Thickened palmar fascia, fixed finger flexion	Alcohol-related liver disease
☐	Asterixis	Arms out, wrists cocked back, fingers spread, ≥20 seconds	Liver failure ("flap")
Arms			
☐	Skin	Bruising, petechiae, scratch marks, spider naevi	Coagulopathy, cholestasis (pruritus), chronic liver disease
☐	Muscle	Wasting	Malnutrition
Face			
☐	Eyes	Pale conjunctiva, scleral jaundice, xanthelasma	Anaemia; liver disease; cholestasis
☐	Salivary glands	Parotid and submandibular enlargement	Alcohol, sialadenitis
☐	Mouth (torch)	Angular stomatitis, dentition, oral candidiasis	Iron/B deficiency, immunosuppression
☐	Lymph nodes*	Left supraclavicular (Virchow's) node †	Gastric malignancy (Troisier's sign)
Chest			
☐	Spider naevi	>5 in SVC distribution †	Chronic liver disease
☐	Gynaecomastia*	Verbalise — not expected to examine	Chronic liver disease, spironolactone
Abdomen — inspection (stand back, then from the side)			
☐	Scars, stoma	Surgical or trauma scars; laparoscopic ports	Previous surgery
☐	Distension	5 Fs: fat, fluid, faeces, flatus, fetus	Ascites, obstruction
☐	Localised swelling	Mass, bowel loop, enlarged organ	
☐	Veins and pulsation	Prominent veins; visible pulsation or peristalsis	Portal hypertension; AAA; obstruction
☐	Cough*	Ask patient to cough — watch hernial orifices and for pain †	Hernia; peritonism
Abdomen — palpation (warm hands, kneel to bed level †, watch face)			
☐	Light palpation	All 9 regions, flat of hand, painful area last	Tenderness, guarding, rigidity, rebound
☐	Deep palpation	All 9 regions again	Masses — describe (see box)
☐	Liver	Start RIF, hand parallel to right costal margin; patient breathes in through mouth; move up 1–2 cm on each breath out	Edge: hard/soft, tender, regular, pulsatile
☐	Spleen	Left hand behind left lower ribs; right hand from RIF towards left costal margin	Any palpable spleen is abnormal
☐	Kidneys	Bimanual ballot each side	Normal kidneys are impalpable
☐	Aorta	Both hands either side above umbilicus	Expansile + pulsatile = AAA

	Step	What to do / say	Abnormal
☐	Bladder*	Palpate suprapubic area if retention suspected	Dull to percussion
Abdomen — percussion			
☐	Liver span	Right MCL from above 6th rib (resonant → dull) down to palpable edge	Normal <12 cm
☐	Spleen*	Percuss left costal margin / Traube's space †	
☐	Shifting dullness	Percuss from midline to flank until dull; mark; roll patient towards you, wait 30 s, re-percuss †	Dullness shifts = ascites
☐	Bladder*	If urinary retention suspected	Dullness above pubis
Abdomen — auscultation			
☐	Bowel sounds	Just below umbilicus	Absent (ileus, peritonitis) / tinkling (obstruction †)
☐	Renal bruits	Either side of midline at level of umbilicus (check BP if present)	Renal artery stenosis
☐	Aortic bruit*	Above umbilicus †	Atherosclerosis, AAA
Hernias and groin			
☐	Hernial orifices	Strain or cough: midpoint of inguinal ligament and lateral to pubic tubercle; examine standing	Inguinal / femoral hernia
☐	Inguinal lymph nodes	Both sides	Lymphadenopathy
Legs*			
☐	Oedema	Ankle and sacral pitting oedema †	Hypoalbuminaemia, liver disease

3. Closing and presenting

☐	Completion: "I would also examine the external genitalia, perform a DRE, and dipstick the urine."
☐	Ask about concerns/questions; check understanding
☐	Cover patient, help them dress, leave them comfortable
☐	Put equipment away; wash hands
☐	Present: "Mr X is comfortable at rest with no peripheral stigmata of chronic liver disease. The abdomen is soft and non-tender with no organomegaly or masses, no shifting dullness, and normal bowel sounds."

4. Quick reference

Mass — describe	
Site	Region
Size, shape	cm
Surface, edge	Regular / irregular
Consistency	Hard / soft
Tenderness	
Mobility	Mobile / fixed; moves with breathing?
Pulsatile?	Expansile = aneurysm
Get above it?	Can't get above liver or spleen
Percussion	Resonant / dull

Scar	Operation
Upper midline	Laparotomy (upper GI)
Right subcostal (Kocher's)	Open cholecystectomy
Right paramedian	Older laparotomy
Lower midline	Laparotomy (lower GI, gynae)
Loin	Nephrectomy
RIF gridiron	Appendicectomy
Iliac fossa J-shape	Kidney transplant
Pfannenstiel	Caesarean, gynae
Umbilical port	Laparoscopic surgery

Spleen vs left kidney †	Spleen	Kidney
Notch	Present	Absent
Get above	No	Yes
Moves with breath	Down and medially	Little
Ballotable	No	Yes
Percussion	Dull	Resonant (bowel in front)

- **Hepatomegaly causes †**: congestion (heart failure), infiltration (fatty liver, malignancy), infection (hepatitis).
- **Splenomegaly causes †**: portal hypertension, haematological malignancy, infection (EBV, malaria).
- **Chronic liver disease signs**: clubbing, leukonychia, palmar erythema, Dupuytren's, spider naevi, gynaecomastia, jaundice, ascites, caput medusae.
- Pitfalls: more than one pillow, not watching the face, palpating tender area first, forgetting hernias.